Docetaxel + pertuzumab + trastuzumab

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Docetaxel + HER2 antibodies

Active medicines: Docetaxel + Pertuzumab + Trastuzumab

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- Docetaxel: dexamethasone 8 mg orally twice daily for 3 days starting the day before treatment; minimum 3 doses pre-treatment unless contraindicated. If oral doses were missed, any 20 mg intravenous (IV) replacement is a consultant decision.
- Reconcile docetaxel premedication with the antiemetic steroid course and interactions; issue one coherent steroid prescription, not duplicate courses.
- Consider primary granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) using the source and individual risk; product and timing follow the prescribed plan.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.
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Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00204 v11 · p5 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-21 · SACTCheck v0.84.16 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Antibody-only continuation

Active medicines: Pertuzumab + Trastuzumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine taxane dexamethasone, chemotherapy granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) or cytotoxic antiemetic course carries forward.
- Review previous reactions and individual rescue needs; keep cardiac checks on the existing clock.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00204 v11 · p5 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-21 · SACTCheck v0.84.16 · Regimen-level drafts; prescriber and oncology-pharmacy verification required